



THE PLAN

Who buys it, and who else is out there

This document tells you who will pay you money, who you call first, and who you are up against.

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

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This document tells you who will pay you money, who you call first, and who you are up against.

Labels used below: **ESTIMATE** (our best guess, not a fact), **KNOWN** (checked and true today), **TO VALIDATE** (must be tested before you say it out loud).

In one minute

- Vytalix has no customers, no pilots and no partners today (KNOWN). Everything here is a plan.
- Only three kinds of buyer can pay you within 90 days: health-tech companies, NGOs with a live grant, and universities or accelerators.
- The NHS, pharma and African governments are worth talking to now, but they pay in 2027 or later.
- MaternaLink is bought by hospitals, NGOs and clinics. It is used by mothers and midwives. Those are different people.
- Nobody sits exactly where you want to sit. But every neighbouring position is taken by someone with more money.

What to do

Action	Who	By when
Spend at least 60% of selling time on health-tech founders and accelerators	Founder	Every week, from day 1
Load 85 target accounts into the CRM, each with a public trigger and its web link	Founder	Day 14
Run 15 UK maternity interviews and 10 Nigeria/Kenya NGO interviews	Founder	Day 60
Replace every ESTIMATE price in the table below with what buyers actually said	Founder	Day 90
Have one honest conversation with mDoc about working alongside them, not against them	Founder	Day 90

Who actually pays

Every price and every timescale below is an ESTIMATE from desk research. None has been tested with a real buyer.

Buyer	What they buy	What they care about	Time to decide (ESTIMATE)	What they can spend (ESTIMATE)
Health-tech companies (pre-seed to Series A, UK or arriving in the UK)	Advice on entering the NHS or Africa; grant and investor readiness	Speed, a fixed price, and a founder who has done it	2–6 weeks	£9,500–£40,000 in year one; or £2,500–£6,000 a month
NGOs and development organisations	Programme design, evaluation, joint grant bids; later a MaternaLink cohort licence	Spending the grant properly and reporting to the donor	3–9 months, tied to grant cycles	£10,000–£50,000 advice; £8,000–£60,000 a year per cohort
Universities and accelerators	Workshops, guest teaching, course licences, being a partner on a grant bid	Teaching content that is current; a company partner for bids	3–9 months (academic year)	£2,500–£6,000 a workshop; £3,000–£8,000 a course licence

Buyer	What they buy	What they care about	Time to decide (ESTIMATE)	What they can spend (ESTIMATE)
Private maternity units and clinics	A short discovery sprint; later a paid MaternaLink pilot	Looking different from the NHS; happy private patients	4–12 weeks UK; 2–4 months Africa	£5,000–£25,000 in year one
Charities and community groups	Paid co-design and listening work	Reaching women the NHS misses; not being used as an endorsement	2–6 months	£3,000–£15,000 (they are credibility, not revenue)
Private hospital groups	Strategy sprints; a part-time strategy officer	References at their scale; one plan across many sites	3–6 months	£14,000–£28,000 a sprint
UK government bodies (ICBs and LMNSs)	Health-inequalities reviews and business cases	Being on an approved supplier framework	3–9 months	£25,000–£150,000, usually through a bigger firm
Employers and benefits platforms	Workshops now; a maternity benefit much later	Keeping women through pregnancy and return to work	4–10 weeks for workshops	£5,000–£15,000
NHS trusts	Readiness and inequalities reviews; later a MaternaLink pilot	Safety paperwork, CQC actions, and not buying another app	3–9 months advice; 9–18 months product	£15,000–£30,000 advice; £0 for a design partnership
Pharmaceutical companies	Programme evaluation and landscape studies	Showing their funded programmes worked	6–12 months, plus 1–3 months to onboard you	£26,000–£80,000
African state governments	Programme design and pilot scoping, paid by a donor	Who is funding it, and where the data is hosted	9–24 months, with real payment risk	£15,000–£80,000 (donor-funded)
Insurers and HMOs	Nothing yet. Interviews only	Proof it changes claims cost	6–12 months, once evidence exists	£0 until a pilot has run
Care and community-worker organisations	Nothing yet. Design input for version 2	One more app for staff who already have three	3–9 months	£3,000–£10,000 design work

Reading the table: the top three rows are the only places a founder with no company, no insurance and no case study can win money this year.

The first ten people to call

These are ten kinds of account, not ten invented names. Each has a sign you can see in public. Put a real account in the CRM only when you have found that sign and recorded the link.

#	The account	The sign to look for	The job title you contact	What you offer first (ESTIMATE)
1	UK health-tech start-up that just raised money and wants the NHS	Funding announcement; a job advert for "NHS partnerships"	Founder or CEO	Grant and investor readiness, £9,500
2	US or EU health-tech company announcing UK entry	Press release; a new UK company at Companies House; first UK hire	UK Country Lead, VP International or COO	UK market-entry playbook, £22,000

#	The account	The sign to look for	The job title you contact	What you offer first (ESTIMATE)
3	UK or EU company or NGO planning Nigeria, Kenya or Ghana	An Africa pilot announced; a "Head of Africa" advertised	CEO or Head of International	Africa market-entry and partner scan, £20,000
4	Accelerator or incubator running a cohort soon	Applications open; a demo day booked; a call for mentors	Programme Director	Cohort workshop, £4,000
5	NGO that won a maternal or digital grant in 2026	Grant announcement; a request for an evaluation partner	Country Director or Programme Manager	Programme design or light evaluation, £10,000–£26,000
6	Private maternity unit launching or refreshing a service	Refurbishment news; a new lead consultant	Hospital Director or Lead Obstetrician	Discovery sprint, £14,000
7	NHS trust with a new midwifery director or a recent CQC report	The appointment; the CQC report	Director of Midwifery; Maternity Digital Lead	A free 30-minute conversation, nothing else in 2026
8	ICB or LMNS with a published maternity equity plan	The board paper; the plan itself	LMNS Programme Director; Health Inequalities Lead	Inequalities review, £28,000, expect to close in 2027
9	Maternity record vendor looking for patient-facing partners	A roadmap announcement; a hospital go-live in the press	VP Partnerships or Product Director	Advisory retainer, £4,000 a month
10	University midwifery or maternal-health research group	An open funding call; a curriculum review; a PhD advert	Principal Investigator or Programme Lead	A free guest lecture, then a £4,000 workshop

Target for 90 days: about 85 contacted, 30 conversations, 10 proposals, 3 signed.

Who else is out there

No relationship exists with any organisation named below (KNOWN). They are listed because they are public and they matter.

Products that do part of this in the UK

Who	What they do	Why they matter to you
BadgerNet (System C)	The maternity record used by many NHS trusts, with a patient app	The biggest threat. If they add more woman-facing features, your UK case gets harder
K2 Medical Systems (Athena, Guardian)	Maternity record plus labour monitoring	Owns the clinical record and the relationship with the trust
Euroking (Magentus)	Older maternity record being replaced in several trusts, 2026–28	Every replacement is a moment when a trust is willing to talk
Baby Buddy (Best Beginnings, a charity)	Free pregnancy information app	Sets the expectation that information is free. Better as a content partner than a rival
MamaDoc (Leicester)	Education, antenatal classes and WhatsApp support for Black, Brown and migrant parents. Led by Dr Yusuf Shittu, an NHS GP, with Dr Nimat	Reaches the women you are talking about and is trusted by them. A partner, not a rival, unless they move into the clinical record

Who	What they do	Why they matter to you
The Motherhood Group (Sandra Igwe)	Black maternal health campaigning, community and training	Runs the conversation this subject happens inside. Influence, not software
NHS App and hospital patient portals	Appointments, letters, results	Trusts will say "women already have an app". Your answer is handover and escalation, not information
Babyscripts (USA, \$37m raised)	Virtual maternity care built into the clinic's workflow	Not in the UK today. A model to learn from, and an investor's comparison
Maven Clinic (USA, \$425m+ raised)	Virtual women's-health clinic sold to employers	Already sells to global employers. Blocks the UK employer route

Products that do part of this in Africa

Who	What they do	Why they matter to you
mDoc (Digital Mom Project, Nigeria)	Coaching, chatbot and hubs; funded by MSD for Mothers; about 24,000 women in Ekiti	The incumbent in the exact state named in the March 2026 proposal. Talk to them, not around them
Jacaranda Health (PROMPTS, Kenya)	Text-message triage for 2.8m+ women at under \$1 per mother	The price anchor. Any price above a few pounds must buy more than messages
MomConnect (South Africa)	National text and WhatsApp service run by government and Praekelt	Proof that a government will run messaging itself, free to the mother
Helium Health (Nigeria, \$42m raised)	Hospital records and financing across West Africa	Owns the facility record. A possible partner; a threat if they add maternity workflow
Totohealth (Kenya, Tanzania)	Text-message information for parents	Small, but shows how cheap the messaging layer is
Zuri Health (Kenya)	Telehealth sold through mobile networks	A route to reach women, not a rival for facility coordination

Consultancies that compete for the advisory work

Who	What they sell that overlaps yours	Typical job size (ESTIMATE)	Your stance
Deloitte	NHS strategy, maternity reviews, donor programme management	£100,000–£5m+	Never compete. Ask for sub-contracted days
PA Consulting	NHS transformation and service redesign	£80,000–£2m	Sub-contract channel. Learn how they write bids
Carnall Farrar	NHS and ICB strategy, maternity and women's health	£50,000–£1m	Your closest UK rival, and the most natural firm to sub-contract you
Health Innovation Networks (15 in England)	Market-access advice to health-tech firms, often free	Free or grant-funded	Both a rival and your best door into NHS trusts
Dalberg	Health-system and digital strategy for ministries and donors	\$100,000–\$2m	Your rival for African donor work; also a possible prime contractor
Genesis Analytics	Health financing and impact evaluation across Africa	\$50,000–\$1m	Rival for evaluation work; a good evaluation partner for a pilot

Your real market is the gap below their minimum: £7,500 to £28,000, fixed price, delivered by you.

What we can honestly claim is different

Each of these is buildable. None can be said out loud until the proof next to it exists.

What is different	Proof needed before you say it	When (ESTIMATE)
A vendor-neutral handover layer that arrives with its safety paperwork done	Papers written, and one hospital safety officer has reviewed them	Month 9–12
Content co-designed and paid for with Black maternal-health and community groups, in several languages	Signed paid agreements; a named clinician signs off the content; 20 women tested it per market	Month 6–12
One product that works both in a London hospital and in a rural area with poor signal	The prototype passes an audit; a field test in a low-signal setting	Month 12–18
Honest evidence: the evaluation and the hazard log are published, and no outcome is claimed	A registered service evaluation with a hospital or NGO	Month 12–18
An Africa model where the NGO or state owns the data and keeps it after you leave	An NGO cohort agreement; a documented conversation with mDoc	Month 9–18
Advice work that feeds the product, and a product that makes the advice credible	Two engagements delivered, with the client's written consent to be named	Month 6–12
A founder who delivers in weeks at a fixed price	Three jobs delivered on time and on price	Month 6

What we cannot claim yet

Say none of these until the second column is true.

Never say this today	What would have to happen first
"Reduces maternal deaths" or "predicts pre-eclampsia"	A research programme, ethics approval, real validation and an MHRA route
"Clinically validated", "NHS-approved", "DTAC-compliant", "CE or UKCA marked"	A hospital assesses it and the safety case is signed
"Government partner" or "working with Ekiti State"	A signed agreement in Vytalix's name, after the ownership of MaternaLink is settled
"Our platform serves X women"	An audited prototype and a live pilot with consenting users
"Cheaper than the others"	Never claim this. The others are free or donor-funded. Compete on what you do, not on price
"Proven AI"	There is a concept deck and nothing else
"UK–Africa corridor experts"	Two delivered jobs across both, and a Nigerian health-system adviser in place
"Endorsed by [any charity or college]"	Written permission from them for a specific sentence
"We own MaternaLink"	A signed assignment or licence of the intellectual property

The £300 per woman per year figure from the March 2026 Ekiti proposal is withdrawn. The 2025 AI prediction deck is withdrawn from outside use.

Words explained

Word	What it means
EPR	Electronic patient record: the hospital's main computer system holding a woman's notes
DTAC	The NHS checklist a digital product must pass before a hospital can buy it
DCB0160 / DCB0129	The NHS safety paperwork: one for the hospital using a system, one for the company making it
CQC	The regulator that inspects and rates hospitals in England
ICB / LMNS	The regional NHS body that plans care, and its maternity planning group
HMO	A Nigerian health insurer that also runs its own care
MHRA	The UK medicines and medical devices regulator
CRM	The spreadsheet or tool where you track every account and every next step